

ENQUIRY SYSTEM STRATEGY

SkinFit Wellness

How an instant first response, the kAI assessment, and a record of every conversation work as one system rather than three tools.

01

Where this picks up

In our last conversation you set out how you see marketing in two parts. Part A is brand, positioning, offers and messaging, which SkinFit is doing the heavy lifting on. Part B is operations and optimisation, which is where we come in.

You were also clear that a full end-to-end build is not the right investment at two months in, that the return on it would be theoretical at this stage, and that you want your front desk learning the conversation on the ground first. We agree, and this document is scoped to that view rather than around it.

What you asked for was narrower and more specific: an instant first response with proper qualification, the kAI assessment brought into that flow, and every conversation recorded and retrievable per lead. That is what follows.

02

The one thing worth naming first

At the sixty to sixty-five enquiries a day you described, arriving on a front desk of two people who are also handling walk-ins and phones, the constraint is not lead volume. It is that a low-priced entry offer brings a lot of people, and nobody can tell which of them is worth a rep's afternoon.

So this is not really a speed system. It is triage. Speed is just where triage has to begin.

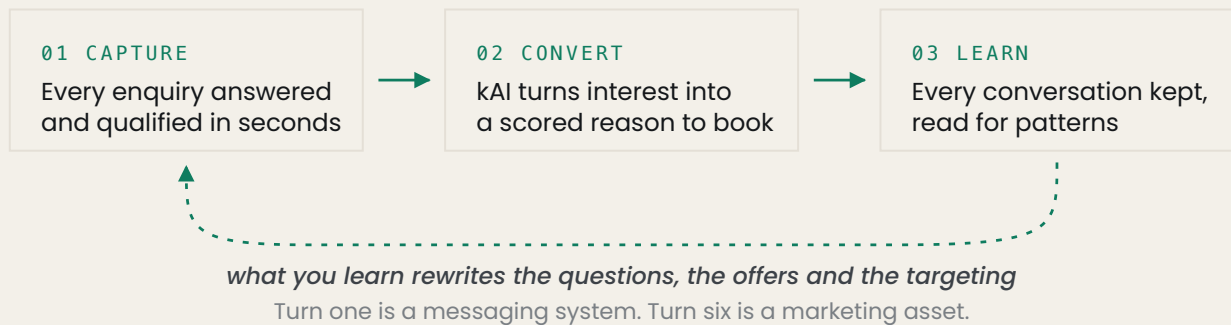
That matters for what gets built. The goal is not to reply faster to sixty people. It is to hand your team a ranked sixty, so the ones worth calling are obvious before anyone picks up the phone.

03

Three jobs, one loop

The three things you asked for are not three separate purchases. They are three stages of the same loop, and the last one feeds the first.

HOW THE PARTS CONNECT



Capture produces structured data at the front: concern, area, how long, urgency.

Convert produces a score in the middle, through kAI.

Learn produces outcome data at the end: what was said, what was objected to, what happened.

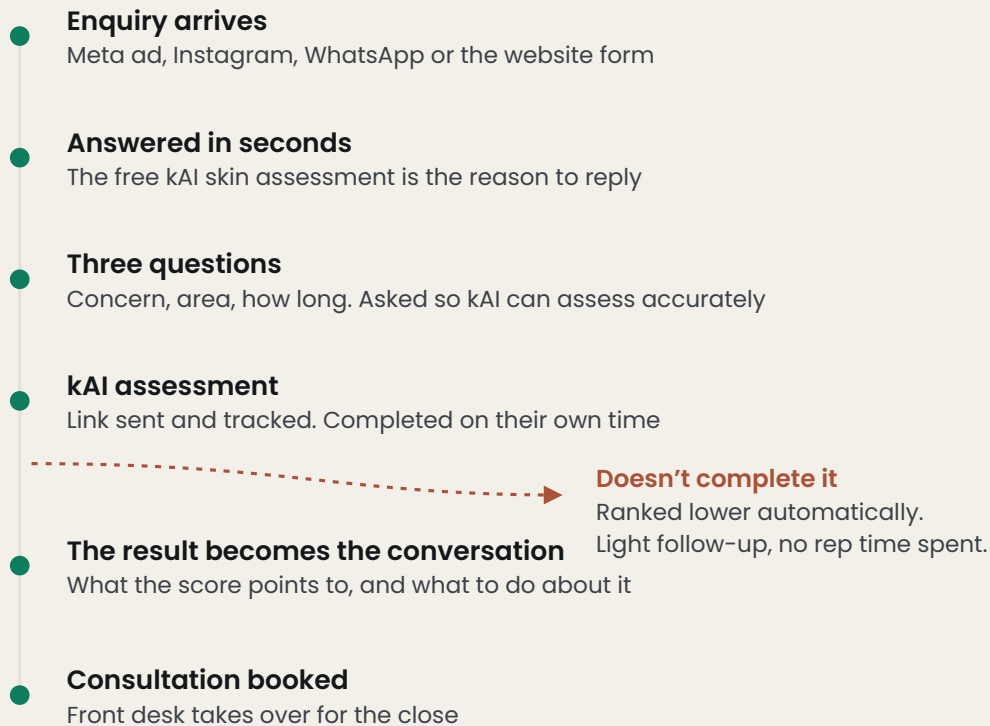
Put together, each lead carries a full record. Which ad brought them, what they were worried about, what kAI found, what they pushed back on, and whether they booked. That record is what rewrites the qualification questions, the way kAI is positioned, and eventually the ads themselves.

04

Bringing kAI into WhatsApp

kAI already ends by pointing people toward an in-person visit, so it is not competing with the consultation. It is the ramp to it. The change is that it stops being something people find on the app, and becomes the reason they answer the first message.

THE FLOW



Why it is built this way

The questions get answered

Asking someone their budget cold loses them. Asking three questions so a free assessment can be done properly gets completed, because it is the price of something they already want.

The close is their own data

When the consultation is offered, it is not a pitch. It is a response to a score they just received, about their own skin.

It sorts itself

Someone who will not spend four minutes on a free assessment was never booking a high-value package. The sixty become a ranked sixty without a rep touching them.

kAI gets used

Right now the assessment sits behind an app download. Here it is offered inside the conversation people are already having with you.

05

The record layer

Every WhatsApp thread and every call tied to the lead it belongs to, so opening a lead shows the full history in one place. That part is straightforward. What it is actually for is the more useful question.

- 01 **The objections, in patients' own words.** Read across a month, the same three or four reasons for hesitating will repeat. Those belong in the ads and the offers, not just in a rep's head.
- 02 **Training material that is real.** Your team is meant to be learning the conversation on the ground. This gives them actual conversations to learn from rather than hypotheticals.
- 03 **Which enquiries are worth paying for.** Tied back to the ad or channel, it shows which traffic produces bookings and which produces conversations that go nowhere.
- 04 **Continuity for the patient.** Nobody re-asks something the person already answered last week. For a clinic building a reputation on experience, that is not a small thing.

06

Why this is not a WhatsApp tool

There are plenty of tools that will send automated WhatsApp replies. They are not the same thing as this, and the difference is worth being precise about.

- 01 **They sell Part B and never touch Part A.** This is the only version of Part B that feeds back into Part A. The record layer is the bridge: what happens in operations becomes input for strategy.
- 02 **They cannot integrate kAI.** Template flows and generic chatbots do not wrap around a clinic's own product. That is custom work by definition.
- 03 **They handle WhatsApp, not calls.** Unifying both into one lead record is a different piece of infrastructure, and almost nobody does it.
- 04 **They hand you a tool, not an outcome.** Somebody at SkinFit would have to build and maintain the flows. That is usually why a chatbot gets tried once and quietly abandoned.
- 05 **They store conversations, they do not read them.** Nobody hands you the objections your patients actually raised this month.

On the return being theoretical

You were right that a full build would be sold on a promise at this stage. This one is not, because the record layer produces something measurable from the first month: response times, the objections coming up most often, and which channels produce booked consultations rather than conversations.

None of that exists today. That is the return, and it is visible well before the volume arrives.

07

What we need to confirm

Four open items. Each one changes how a part of this gets built, so they are settled before anything is committed rather than assumed now.

- 01 **Whether kAI can return a result to us.** If the assessment outcome can come back into the conversation, the flow closes cleanly and the follow-up is built on the actual score. If it is a one-way link, the flow still works but the handoff after the assessment is shaped differently.
- 02 **Which lines the calls run on.** Transcription depends on where calls actually arrive. Personal mobiles, a clinic landline, or a telephony service are three different builds, and this is the largest single variable in the whole scope.
- 03 **What the sixty to sixty-five counts.** Whether that is every conversation started, or the ones already filtered as worth pursuing. It changes what the qualification layer is actually sorting.
- 04 **Where the front desk works.** Whether the ranked lead and its history should appear in a system they already use, or somewhere new.

Call recording needs a consent line to the patient before the recording starts, and the retention terms in the Scope of Work will be updated to reflect a stored conversation history. **Both are settled before anything is switched on, not after.**

08

What happens next

This document covers the shape, not the commercials. Two of the three parts, the kAI integration and the record layer, were not in anything quoted previously, so they need our technical team to scope them properly before a number is put in front of you.

If the strategy above is the right one, we will confirm the four items in Section 07 and come back with one fixed quotation for exactly this scope, not the wider build.

The first system to go live would be the instant response and qualification layer, since kAI and the record layer both sit on top of it. **That order gets agreed together before work starts.**